

Lumbar Radiculopathy — Clinical Pathway

Apex Specialist Centre · Pathway Reference: ORTHO-SPINE-04 · Version 4.2 · Effective 2026-01-01

1. Scope

This pathway applies to adult patients (≥ 18 years) presenting with unilateral lumbar radiculopathy. It does NOT apply to: cauda equina syndrome (emergency referral), progressive neurological deficit (urgent surgical review), suspected malignancy, infection, or trauma.

2. Red-Flag Triage (do these BEFORE entering the pathway)

- Saddle anaesthesia / bladder or bowel dysfunction → ED, exclude cauda equina
- Bilateral leg weakness or rapidly progressive deficit → urgent surgical review
- Weight loss, night pain, history of malignancy → urgent imaging + oncology workup
- Fever, recent procedure, IVDU → suspect discitis / epidural abscess → urgent MRI + ID

3. Conservative Phase (Weeks 0–6)

First-line: analgesic ladder (paracetamol → NSAIDs → neuropathic agents), structured physiotherapy 2x/week, activity modification. No bed rest. Re-assess at 6 weeks.

4. Imaging

MRI lumbar spine is the imaging of choice if: symptoms persist >6 weeks, neurological deficit present, OR red flags identified.

5. Treatment Options (after imaging confirms disc pathology)

Option A — Continued Conservative: for patients with stable or improving symptoms, no progressive deficit. Continue physio + analgesia for further 6 weeks. Indicated when imaging shows protrusion ≤ 7 mm and SLR positive but no motor weakness >grade 1.

Option B — Targeted Epidural Steroid Injection (ESI): for patients with significant radicular pain unresponsive to oral medication, imaging-confirmed nerve root contact, NO motor weakness progression. Contraindicated in: uncontrolled diabetes (HbA1c >8.5%), anticoagulation, active infection.

Option C — Surgical Decompression (microdiscectomy): indicated when (1) progressive neurological deficit, OR (2) failure of 6-12 weeks of conservative + ESI, AND (3) imaging-clinical correlation clear. Contraindications: HbA1c >9.0% (delayed wound healing), active systemic infection, severe cardiac comorbidity.

Option D — Multidisciplinary Pain Programme: for chronic radiculopathy >12 weeks not amenable to surgery. CBT + physio + pain medicine.

6. MDT Decision

Patients NOT meeting clear surgical criteria must be presented at the Spinal MDT (Wednesdays 14:00) before progressing to any invasive treatment. Required at MDT: history summary, imaging report, recent labs (HbA1c, FBC, U&E, CRP), patient preference / consent discussion.

7. Outcomes & Follow-Up

- Conservative: review week 6 and 12, discharge if pain VAS ≤ 2 sustained
- ESI: review 4 weeks post-injection

- Surgical: review 2 weeks, 6 weeks, 3 months post-op
- All patients: lifestyle counselling, weight management, glycaemic optimisation as relevant